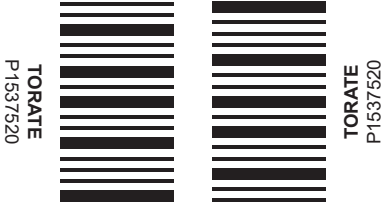


Pharmacode position may change as per Supplier’s m/c requirement & additional small pharma code may appear on the front / back panel



TORATE
50 mg & 100 mg Tablets
Topiramate tablets 50 mg & 100 mg

Description
TORATE 50MG TABLETS
Light yellow colored, circular, biconvex film-coated tablets debossed with ‘E’ on one side and ‘33’ on the other side. Each film-coated tablet contains topiramate 50mg.

TORATE 100MG TABLETS
Dark yellow colored, circular, biconvex beveled edged film-coated tablets debossed with ‘E’ on one side and ‘23’ on the other side. Each film tablet contains Topiramate 100 mg.

Pharmacodynamics
Topiramate is classified as a sulfamate-substituted monosaccharide. The precise mechanisms by which topiramate exerts its antiseizure and migraine prophylaxis effects are unknown.

Electrophysiological and biochemical evidence suggests that topiramate, at pharmacologically relevant concentrations, blocks voltage-dependent sodium channels, augments the activity of the neurotransmitter gamma-aminobutyrate at some subtypes of the GABA-A receptor, antagonizes the AMPA/kainate subtype of the glutamate receptor, and inhibits the carbonic anhydrase enzyme, particularly isozymes II and IV.

Pharmacokinetics
The pharmacokinetic profile of topiramate compared to other antiepileptic drugs shows along plasma half-life, linear pharmacokinetics, predominantly renal clearance, and absence of significant protein binding and lack of clinically relevant active metabolites.

Topiramate is not a potent inducer of drug metabolizing enzymes, can be administered without regard to meals, and routine monitoring of plasma topiramate concentrations is not necessary. In clinical studies, there was no consistent relationship between plasma concentrations and efficacy or adverse events.

Indication
1. TORATE is indicated as monotherapy in patients with newly diagnosed epilepsy or for conversion to monotherapy in patients with epilepsy.
2. TORATE is indicated as adjunctive therapy for adults and children 2 years and above with partial onset seizures or generalized tonic-clonic seizures.
3. TORATE is also indicated in adults and children as adjunctive therapy for the treatment of seizures associated with Lennox Gastaut syndrome.
4. TORATE is indicated in adults for the prophylaxis of migraine headache. The usefulness of TORATE in the acute treatment of migraine headache has not been studied.

Recommended dose
General
It is recommended that therapy be initiated at a low dose followed by titration to an effective dose. TORATE is available in tablets. It is recommended that tablets not be broken.

It is not necessary to monitor topiramate plasma concentrations to optimize therapy with TORATE. On rare occasions, the addition of TORATE to phenytoin may require an adjustment of the dose of phenytoin to achieve optimal clinical outcome. Addition or withdrawal of phenytoin and carbamazepine to adjunctive therapy with TORATE may require adjustment of the dose of TORATE.

TORATE can be taken without regard to meals.

Dosage
It is recommended that therapy be initiated at a low dose followed by titration to an effective dose.

Adjunctive Therapy Epilepsy
Adults
Therapy should begin at 25-50 mg nightly for one week. Use of lower initial doses has been reported, but has not been studied systematically. Subsequently, at weekly or-biweekly intervals, the dose should be increased by 25-50 [to 100] mg/day and taken in two divided doses. Dose titration should be guided by clinical outcome. Some patients may achieve efficacy with once-a- day dosing. These dosing recommendations apply to all adults, including the elderly, in the absence of underlying renal disease.

Children aged 2 and above
The recommended total daily dose of TORATE (topiramate) as adjunctive therapy is approximately 5 to 9 mg/kg/day in two divided doses. Titration should begin at 25 mg (or less, based on a range of 1 to 3 mg/kg/day) nightly for the first week. The dosage should then be increased at 1- or 2-week intervals by increments of 1 to 3 mg/kg/day (administered in two divided doses), to achieve optimal clinical response. Dose titration should be guided by clinical outcome.

Daily doses up to 30 mg/kg/day have been studied and were generally well tolerated.

Monotherapy Epilepsy
General
When concomitant AEEDs are withdrawn to achieve monotherapy with topiramate, consideration should be given to the effects this may have on seizure control. Unless safety concerns require an abrupt withdrawal of the concomitant AED, a gradual discontinuation at the rate of approximately one-third of the concomitant AED dose every 2 weeks is recommended.

When enzyme inducing drugs are withdrawn, topiramate levels will increase. A decrease in TORATE dosage may be required if clinically indicated.

Adults
Titration should begin at 25 mg nightly for 1 week. The dosage should then be increased at 1- or 2- week intervals by increments of 25 or 50 mg/day, administered in two divided doses. If the patient is unable to tolerate the titration regimen, smaller increments or longer intervals between increments can be used. Dose and titration rate should be guided by clinical outcome.

The recommended initial target dose for topiramate monotherapy in adults is 100 mg/day and the maximum recommended daily dose is 500 mg. some patients with refractory forms of epilepsy have tolerated topiramate monotherapy at doses of 1000 mg/day. These dosing recommendations apply to all adults including the elderly in the absence of underlying renal disease.

Children
Treatment of children aged 2 years and above should begin at 0.5 to 1 mg/kg nightly for the first week. The dosage should then be increased at 1- or 2- week intervals by increments of 0.5 to 1 mg/kg/day, administered in two divided doses. If the child is unable to tolerate the titration regimen, smaller increments or longer intervals between dose increments can be used. dose and dose titration rate should be guided by clinical outcome.

The recommended initial target dose range for topiramate monotherapy in children aged two years and above is 3 to 6 mg/kg/day. Children with recently diagnosed partial onset seizures have received doses of up to 500 mg/day.

Migraine
Adult
The recommended total daily dose of topiramate for prophylaxis of migraine headache is 100 mg/day administered in two divided doses. Titration should begin at 25 mg nightly for 1 week. The dosage should then be increased in increments of 25 mg/day administered at 1-week intervals. If the patients in unable to tolerate the titration regimen, longer intervals between dose adjustments can be used. Some patients may experience a benefit at a total daily dose of 50 mg/day. Patients have received a total daily dose up to 200 mg/day. Dose and titration rate should be guided by clinical outcome.

Special populations
Renal impairment
Patients with moderate and severe renal impairment (CLCR <70 mL/min) may require a dose reduction. Half of the usual starting and maintenance dose is recommended.

Since TORATE is removed from plasma by hemodialysis, a supplemental dose of TORATE equal to approximately one-half the daily dose should be administered on hemodialysis days. The supplemental dose should be administered in divided doses at the beginning and completion of the hemodialysis procedure. The supplemental dose may differ based on the characteristics of the dialysis equipment being used.

Hepatic impairment
Topiramate should be administered with caution in patients with hepatic impairment.

Contraindication
Hypersensitivity to any component of this product.

Warning and precautions
Withdrawal with TORATE
In patients with or without a history of seizures or epilepsy, antiepileptic drugs, including TORATE should be withdrawn gradually to minimize the potential for seizures or increased seizure frequency.

Renal Impairment
The major route of elimination of unchanged topiramate and its metabolites is via the kidney. Renal elimination is dependent on renal function and is independent of age. Patients with moderate or severe renal impairment may take 10 to 15 days reach steady state plasma concentrations as compared to 4 to 8 days in patients with normal renal function.

As with all patients, the titration schedule should be guided by clinical outcome (i.e., seizure control, avoidance of side effects) with the knowledge that subjects with known renal impairment may require a longer time to reach steady-state at each dose.

Hydration
Adequate hydration while using topiramate is very important. Hydration can reduce the risk of nephrolithiasis (see below). Proper hydration prior to and during activities such as exercise or exposure to warm temperatures may reduce the risk of heat-related adverse reactions.

Mood disturbances/depression
An increased incidence of mood disturbances and depression has been observed during topiramate treatment.

Suicide/suicide ideation
Potential for an increase in risk of suicidal thoughts or behaviors.

Antiepileptic drugs (AEDs), including TORATE, increase the risk of suicidal thoughts or behavior in patients taking these drugs for any indication. The mechanism of this risk is not known.

Patients therefore should be monitored for signs of suicidal ideation and behavior and appropriate treatment should be considered. Patients (and caregivers of patients) should be advised to seek medical advice should signs of suicidal ideation or behavior emerge.

Serious skin reactions
It is recommended that patients be informed about the signs of serious skin reactions. If Stevens Johnson Syndrome (SJS) or Toxic Epidermal Necrolysis TEN are suspected, use of TORATE should be discontinued.

Nephrolithiasis
Some patients, especially those with a predisposition to nephrolithiasis, may be at increased risk for renal stone formation and associated signs and symptoms such as renal colic, renal pain or flank pain.

Risk factors for nephrolithiasis include prior stone formation, a family history of nephrolithiasis and hypercalciuria. None of these risk factors can reliably predict stone formation during topiramate treatment. In addition, patients taking other medicinal products associated with nephrolithiasis may be at increased risk.

Hepatic impairment
In hepatically-impaired patients, topiramate should be administered with caution as the clearance of topiramate may be decreased.

Acute myopia and secondary angle closure glaucoma
A syndrome consisting of acute myopia associated with secondary angle closure glaucoma has been reported in patients receiving topiramate. Symptoms include acute onset of decreased visual acuity and/or ocular pain. Ophthalmologic findings can include myopia, anterior chamber shallowing, ocular hyperaemia (redness) and increased intraocular pressure. Mydriasis may or may not be present. This syndrome may be associated with supraciliary effusion resulting in anterior displacement of the lens and iris, with secondary angle closure glaucoma. Symptoms typically occur within 1 month of initiating topiramate therapy. In contrast to primary narrow angle glaucoma, which is rare under 40 years of age, secondary angle closure glaucoma associated with topiramate has been reported in paediatric patients as well as adults. Treatment includes discontinuation of topiramate, as rapidly as possible in the judgment of the treating physician, and appropriate measures to reduce intraocular pressure. These measures generally result in a decrease in intraocular pressure.

Elevated intraocular pressure of any aetiology, if left untreated, can lead to serious sequelae including permanent vision loss.

Visual field defects
Visual field defects have been reported in patients receiving topiramate independent of elevated intraocular pressure. In clinical trials, most of these events were reversible following topiramate discontinuation, however some cases were not. In a large proportion of post marketing case reports reversibility was unknown, but in cases where an outcome was reported, the majority was reversible. If visual problems occur at any time during topiramate treatment, consideration should be given to discontinuing the drug.

Metabolic acidosis and sequelae
Hyperchloremic, non-anion gap, metabolic acidosis (i.e. decreased serum bicarbonate below the normal reference range in the absence of respiratory alkalosis) is associated with topiramate treatment. This decrease in serum bicarbonate is due to the inhibitory effect of topiramate on renal carbonic anhydrase. Generally, the decrease in bicarbonate occurs early in treatment although it can occur at any time during treatment.

These decreases are usually mild to moderate (average decrease of 4 mmol/l at doses of 100 mg/day or above in adults and at approximately 6 mg/kg/day in paediatric patients). Rarely, patients have experienced decreases to values below 10 mmol/l. Conditions or therapies that predispose to acidosis (such as renal disease, severe respiratory disorders, status epilepticus, diarrhoea, surgery, ketogenic diet or certain medicinal products) may be additive to the bicarbonate lowering effects of topiramate.

Chronic, untreated metabolic acidosis may increase the risk of nephrocalcinosis.

Chronic metabolic acidosis increases the risk of renal stone formation and may potentially lead to osteopenia. Chronic metabolic acidosis in paediatric patients can reduce growth rates. The effect of topiramate on bone- related sequelae has not been systematically investigated in paediatric or adult populations.

Depending on underlying conditions, appropriate evaluation including serum bicarbonate levels is recommended with topiramate therapy. If metabolic acidosis develops and persists, consideration should be given to reducing the dose or discontinuing Topiramate (using dose tapering).

Hyperammonaemia and encephalopathy
The risk for hyperammonemic with topiramate appears dose-related. Hyperammonemic has been reported more frequently when topiramate is used concomitantly with valproic acid. Clinical symptoms of hyperammonemic encephalopathy often include acute alterations in level of consciousness and/or cognitive function with lethargy. In most cases, hyperammonemic encephalopathy abated with discontinuation of treatment. In patients who develop unexplained lethargy, or changes in mental status associated with topiramate monotherapy or adjunctive therapy, it is recommended to consider hyperammonemic encephalopathy and measuring ammonia levels.

Women of childbearing potential
TORATE may cause fetal harm when administered to a pregnant woman. There is an increased risk of pre-term labour and premature delivery associated with the use of AEDs, including topiramate.

TORATE should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus (see Pregnancy and Breast-feeding).

Nutritional supplementation
A dietary supplement or increased food intake may be considered if the patient is losing weight while on this medication.

Interaction with other Medicaments
(For purpose of this section, a no effect dose is defined as a ≤ 15% change)

Effects of other AEDs on TORATE
Phenytoin and carbamazepine decrease the plasma concentration of TORATE. The addition or withdrawal of phenytoin or carbamazepine to TORATE therapy may require an adjustment in dosage of the latter. This should be done by titrating to clinical effect. The addition or withdrawal of valproic acid does not produce clinically significant changes in plasma concentrations of TORATE and, therefore, does not warrant dosage adjustment of TORATE.

Effects of other antiepileptic drugs on TORATE
Phenytoin and carbamazepine decrease the plasma concentration of topimarate. The addition or withdrawal of phenytoin or carbamazepine to TORATE therapy may require an adjustment in dosage of the latter. This should be done by titrating to clinical effect.

The addition or withdrawal of valproic acid does not produce clinically significant changes in plasma concentrations of TORATE and, therefore, does not warrant dosage adjustment of TORATE. The results of these interactions are summarized below:

AED Coadministered	AED Concentration	TORATE Concentration
Phenytoin	↔**	↓ (48%)
Carbamazepine (CBZ)	↔	↓ (48%)
Valproic acid	↔	↔
Lamotrigine	↔	↔
Phenobarbital	↔	NS
Primidone	↔	NS

↔ = No effect on plasma concentration (≤ 15% change)
** = Plasma concentrations increase in individual patients
↓ = Plasma concentrations decrease
NS = Not studied
AED = antiepileptic drug

Other drug interactions
Digoxin
In a single-dose study, serum digoxin area under plasma concentration curve (AUC) decreased 12% due to concomitant administration of TORATE. The clinical relevance of this observation has not been established. When TORATE is added or withdrawn in patients on digoxin therapy, careful attention should be given to the routine monitoring of serum digoxin.

CNS depressants
Concomitant administration of TORATE and alcohol or other CNS depressant medicinal products has not been evaluated in clinical studies. It is recommended that TORATE not be used concomitantly with alcohol or other CNS depressant drugs.

Oral contraceptives
The possibility of decreased contraceptive efficacy and increased breakthrough bleeding should be considered in patients taking combination oral contraceptive products with TORATE. Patients taking estrogen containing contraceptives should be asked to report any change in their bleeding patterns.

Contraceptive efficacy can be decreased even in the absence of breakthrough bleeding.

Lithium
In patients with bipolar disorder, the pharmacokinetics of lithium were unaffected during treatment with topiramate at doses of 200 mg/day; however, there was an observed increase in systemic exposure (26% for AUC) following topiramate doses of up to 600 mg/day. Lithium levels should be monitored when co- administered with topiramate.

Risperidone
There were no significant changes in the systemic exposure of the risperidone total active moiety or of topiramate.

Hydrochlorothiazide (HCTZ)
The addition of HCTZ to topiramate therapy may require an adjustment of the Topiramate dose. The decrease in serum potassium is greater when HCTZ and Topiramate administered in combination.

Metformin
When TORATE is added or withdrawn in patients on metformin therapy, careful attention should be given to the routine monitoring for adequate control of their diabetic disease state.

Pioglitazone
When TORATE is added to pioglitazone therapy or pioglitazone is added to TORATE therapy, careful attention should be given to the routine monitoring of patients for adequate control of their diabetic disease state.

Glyburide
When topiramate is added to glyburide therapy or glyburide is added to Topiramate therapy, careful attention should be given to the routine monitoring of patients for adequate control of their diabetic disease state.

Other forms of interactions
Agents predisposing to nephrolithiasis
TORATE, when used concomitantly with other agents predisposing to nephrolithiasis, may increase the risk of nephrolithiasis. While using TORATE, agents like these should be avoided since they may create a physiological environment that increases the risk of renal stone formation.

Valproic acid
Concomitant administration of topiramate and valproic acid has been associated with hyperammonemia with or without encephalopathy in patients who have tolerated either medicinal product alone. In most cases, symptoms and signs abated with discontinuation of either medicinal product. This adverse reaction is not due to a pharmacokinetic interaction. Hypothermia, defined as an unintentional drop in body core temperature to <35°C, has been reported in association with concomitant use of topiramate and valproic acid (VPA) both in conjunction with hyperammonemia and in the absence of hyperammonemia. This adverse event in patients using concomitant topiramate and valproate can occur after starting topiramate treatment or after increasing the daily dose of topiramate.

Vitamin K-antagonist anticoagulant medications
Decreased Prothrombin Time/International Normalized Ratio (PT/INR) responses have been reported following concomitant administration of topiramate with vitamin K-antagonist anticoagulant medications. Closely monitor INR during concomitant administration of topiramate therapy with vitamin K-antagonist anticoagulant medications.

Additional pharmacokinetic drug interaction between topiramate and other agents
The changes in C_{max} or AUC as a result of the interactions are summarized below. The second column (concomitant drug concentration) describes what happens to the concentration of the concomitant drug listed in the first column when topiramate is added. The third column (topiramate concentration) describes how the coadministration of a drug listed in the first column modifies the concentration of topiramate.

Concomitant Drug	Concomitant Drug Concentration ^a	Topiramate Concentration ^a
Amtripytiline	↔ 20% increase in C _{max} and AUC of nortriptyline metabolite	NS
Dihydroergotamine (Oral and Subcutaneous)	↔	↔
Haloperidol	↔31% increase in AUC of the reduced metabolite	NS
Propranolol	↔ 17% increase in C _{max} for 4- OH propranolol (TPM 50 mg q 12h)	9% and 16% increase in C _{max} 9% and 17% increase in AUC (40 and 80 mg propranolol q 12h respectively).
Sumatriptan (Oral and Subcutaneous)	↔	NS
Pizotifen	↔	↔
Diltiazem	25% decrease in AUC of Diltiazem and 18% decrease in DEM, and ↔ for DEM*	20% increase in AUC
Venlafaxine	↔	↔
Flunarizine	16% increase in AUC (TPM 50 mg q12h) ^b	↔

a % values are the changes in treatment mean C_{max} or AUC with respect to monotherapy
↔ = No effect on C_{max} and AUC (≤ 15% change) of the parent compound NS = Not studied
*DEA = des acetyl diltiazem, DEM = N-demethyl diltiazem
b Flunarizine AUC increased 14% in subjects taking flunarizine alone.
Increase in exposure may be attributed to accumulation during achievement of steady state.

Pregnancy and Lactation

Pregnancy
There are no adequate and well-controlled studies with TORATE in pregnant women. TORATE can cause fetal harm when administered to a pregnant woman.

Pregnancy registry data suggest that there may be an association between the use of TORATE during pregnancy and congenital malformations (e.g., craniofacial defects, such as cleft lip/palate, hypospadias, and anomalies involving various body systems). This has been reported with topiramate monotherapy and topiramate as part of a polytherapy regimen.

In addition, data from these registries and other studies suggest that, compared with monotherapy, there may be an increased risk of teratogenic effects associated with the use of antiepileptic drugs in combination therapy.

TORATE should be used during pregnancy only if potential benefit justifies the potential risk to the fetus. In treating and counseling women of childbearing potential, the prescribing physician should weigh the benefits of therapy against the risks. If this drug is used during pregnancy or if the patient becomes pregnant while taking this drug, the patient should be apprised of the potential hazard to the fetus.

Lactation (Breast-feeding)

The excretion of topiramate in human milk has not been evaluated. Limited observations in patients suggests an extensive excretion of topiramate into breast milk. Since many drugs are excreted in human milk, a decision should be made whether to discontinue nursing or to discontinue the drug taking into account the importance of the drug to the mother.

Effects on ability to drive and use machines

Topiramate acts on the central nervous system and may produce drowsiness, dizziness or other related symptoms. It may also cause visual disturbances and/or blurred vision.

These adverse reactions could potentially be dangerous in patients driving a vehicle or operating machinery, particularly until such time as the individual patient's experience with the medicinal products established.

Side effects

The majority of all adverse reactions were mild to moderate in severity.
The most common ADRs include: anorexia, decreased appetite, bradyphrenia, depression, expressive language disorder, insomnia, coordination abnormal, disturbance in attention, dizziness, dysarthria, dysgeusia, hypoaesthesia, lethargy, memory impairment, nystagnus, paresthesia, somnolence, tremor, diplopia, vision blurred, diarrhoea, nausea, fatigue, irritability, and weight decreased.

Topiramate Adverse Drug Reactions –

System organ class	Very Common	Common	Uncommon	Rare	Very Rare	Not Known
Investigations	Weight decreased	Weight increased*	Crystal urine present, tandem gait test abnormal, white blood cell count decreased	Blood bicarbonate decreased		
Cardiac disorders			Bradycardia, sinus bradycardia, palpitations			
Blood and lymphatic system disorders		Anaemia	Leucopenia, thrombocytopenia, lymphadenopathy, eosinophilia	Neutropenia*		
Nervous system disorders	Paraesthesia, somnolence Dizziness	Disturbance in attention, memory impairment, amnesia, cognitive disorder, mental impairment, psychomotor skills impaired, convulsion, coordination abnormal, tremor, lethargy, hypoaesthesia, nystagnus, dysgeusia, balance disorder, dysarthria,	Depressed level of consciousness, grand mal convulsion, visual field defect, complex partial seizures, speech disorder, psychomotor hyperactivity, syncope, sensory disturbance, drooling, hypersomnia, aphasia, repetitive speech, hypokinesia, dyskinesia, dizziness postural, poor-quality sleep, burning sensation,	Apraxia, circadian rhythm sleep disorder, hyperaesthesia, hyposmia, anosmia, essential tremor, akinesia, unresponsive to stimuli		
		intention tremor, sedation,	sensory loss, parosmia, cerebellar syndrome, dysaesthesia, hypoguesia, stupor, clumsiness, aura, ageusia, dysgraphia, dysphasia, neuropathy peripheral, presyncope, dystonia, formication			
Eye disorders		Vision blurred, diplopia, visual disturbance	Visual acuity reduced, scotoma, myopia*, abnormal sensation in eye*, dry eye, photophobia, blepharospasm, lacrimation increased, photopsia, mydriasis, presbyopia	Blindness unilateral, blindness transient, glaucoma, accommodation disorder, altered visual depth perception, scintillating scotoma, eyelid oedema*, night blindness, amblyopia		Angle closure glaucoma*, Maculopathy*, eye movement disorder* Uveitis (Post marketing data)
Ear and labyrinth disorders		Vertigo, tinnitus, ear pain	Deafness, deafness unilateral, deafness neurosensory, ear discomfort, hearing impaired			
Respiratory, thoracic and mediastinal disorders		Dyspnoea , epistaxis, nasal congestion, rhinorrhoea	Dyspnoea exertional, Paranasal sinus hypersecretion, dysphonia			
Gastrointestinal disorders	Nausea, diarrhoea	Vomiting, constipation, abdominal pain upper, dyspepsia, abdominal pain, dry mouth, stomach discomfort, paraesthesia oral, gastritis, abdominal discomfort	Pancreatitis, flatulence, gastrooesophageal reflux disease, abdominal pain lower, hypoaesthesia oral, gingival bleeding, abdominal distension, epigastric discomfort, abdominal tenderness, salivary hypersecretion,			
			oral pain, breath odour, glossodynia			

Renal and urinary disorders		Nephrolithiasis, pollakiuria, dysuria	Calculus urinary, urinary incontinence, haematuria, incontinence, micturition urgency, renal colic, renal pain	Calculus ureteric, renal tubular acidosis* Nephrolithiasis	Nephro-calcinosis	
Skin and subcutaneous tissue disorders		Alopecia, rash, pruritus	Anhidrosis, hypoaesthesia facial, urticaria, erythema, pruritus generalised, rash macular, skin discolouration, dermatitis allergic, swelling face	Stevens-Johnson syndrome* erythema multiforme*, skin odour abnormal, periorbital oedema*, urticaria localised		Toxic epidermal necrolysis
Musculoskeletal and connective tissue disorders		Arthralgia, muscle spasms, myalgia, muscle twitching, muscular weakness, musculoskeletal chest pain	Joint swelling*, musculoskeletal stiffness, flank pain, muscle fatigue	Limb discomfort*		
Metabolism and nutrition disorders		Anorexia, decreased appetite	Metabolic acidosis, Hypokalaemia, increased appetite, polydipsia	Acidosis hyperchloraemic		
Infections and infestations	Nasopharyngitis*					
Vascular disorders			Hypotension, orthostatic hypotension flushing, hot flush,	Raynaud's phenomenon		
General disorders and administration site conditions	Fatigue	Pyrexia, asthenia, irritability, gait disturbance, feeling abnormal, malaise	Hyperthermia, thirst, influenza like illness*, sluggishness, peripheral coldness, feeling drunk, feeling jittery	Face oedema, calcinosis		
Social circumstances			Learning disability			
Immune system disorders		Hypersensitivity				Allergic oedema*, conjunctival oedema*
Reproductive system and breast disorders			Erectile dysfunction, sexual dysfunction			
Psychiatric disorders	Depression	Bradyphrenia, insomnia, expressive language disorder, anxiety, confusional state, disorientation, aggression, mood altered, agitation, mood swings, depressed mood, anger, abnormal behaviour	Suicidal ideation, suicide attempt, hallucination, Psychotic disorder, hallucination auditory, hallucination visual, apathy, lack of spontaneous speech, sleep disorder, affect lability, libido decreased, restlessness, crying, dysphemia, euphoric mood, paranoia, perseveration, panic attack, tearfulness, reading disorder, initial insomnia, flat affect, thinking abnormal, loss of libido, listless, middle insomnia, distractibility, early morning awakening, panic reaction, elevated mood	Mania, anorgasmia, panic disorder, disturbance in sexual arousal, feeling of despair*, orgasm abnormal, hypomania, orgasmic sensation decreased		

Paediatric population
ADRs reported more frequently (> 2-fold) in children than in adults include: decreased appetite, increased appetite, acidosis hyperchloraemic, hypokalaemia, abnormal behaviour, aggression, apathy, initial insomnia, suicidal ideation, disturbance in attention, lethargy, circadian rhythm sleep disorder, poor quality sleep, lacrimation increased, sinus bradycardia, feeling abnormal, and gait disturbance.

ADRs that were reported in children but not in adults include: eosinophilia, psychomotor hyperactivity, vertigo, vomiting, hyperthermia, pyrexia, and learning disability.

Symptoms and Treatment Overdose Signs and symptoms
Overdoses of topiramate have been reported. Signs and symptoms included convulsions, drowsiness, speech disturbances, blurred vision, diplopia, impaired mentation, lethargy, abnormal coordination, stupor, hypotension, abdominal pain, agitation, dizziness and depression. The clinical consequences were not severe in most cases, but deaths have been reported after overdoses with multiple medicinal products including topiramate.

Topiramate overdose can result in severe metabolic acidosis.

Treatment
In acute topiramate overdose, if the ingestion is recent, the stomach should be emptied immediately by lavage or by induction of emesis. Activated charcoal has been shown to adsorb topiramate in vitro.

Treatment should be appropriately supportive and the patient should be well hydrated. Haemodialysis has been shown to be an effective means of removing topiramate from the body. The patient should be well hydrated.

Storage Condition
Store in a dry place below 30C.

Shelf life
24 months

Presentation
TORATE 50MG TABLETS are to be packed in white opaque round 40 CC HDPE container with 33 mm neck finish closed with white opaque polypropylene closure with induction sealing wad. Each HDPE bottle shall contain one silica gel sachet of 1g .i.e., HDPE Bottle Pack of 60's Count.

TORATE 50 MG TABLETS are to be packed in Blister pack of 30's count and 60's count.
TORATE 100MG TABLETS are to be packed in white opaque round 40 CC HDPE container with 33 mm neck finish closed with white opaque polypropylene closure with induction sealing wad. Each HDPE bottle shall contain one silica gel sachet of 1g .i.e., HDPE Bottle Pack of 60's or 100's Count
TORATE 100 MG TABLETS are to be packed in Blister pack of 30's count and 60's count.

Manufacturer
M/s Aurobindo Pharma Limited Unit III, Survey No. 313 - 314, Bachupally Village, Bachupally Mandal, Medchal-Malkajigiri District, Hyderabad, Telangana, 500090 INDIA.

Product Registration Holder
UNIMED SDN BHD
No. 53, Jalan Tembaga SD 5/2B, Bandar Sri Damansara, 52200, Kuala Lumpur, Malaysia.

Revised date: July 2024

		Product Name	Component	Item Code	Date & Time	
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		Customer / Country	Version No.	Reason of Issue	No. of Colours : 1	
Team Leader		APL_Malaysia_Unit-03	00	New	Black	
Initiator		Dimensions	Font Type	Font Size		
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Date - Designer - Run: 1. 27-JUL-2024 (Adi) - FR 8. 15. 2. 29-JUL-2024 (Adi) - FR 9. 16. 3. 10. 17. 4. 11. 18. 5. 12. 19. 6. 13. 20. 7. 14.				Reviewed / Approved by		
Additional Information: Supercede Item Code: NA				Sign / Date		